

Medicine

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Contents

Introduction	iii
0.1 Organisation	iii
0.2 Suggested chapter structure	iii
0.3 Formatting conventions	iv
0.4 Sources and maintenance	v
 1 Administration	 1
1.1 Important links	1
1.2 Registering with Learning@Wales	1
1.3 Required training	1
1.4 Canvas training	2
1.5 Resources	2
 2 Anatomy	 3
2.1 2026-09-03	3
2.1.1 Anatomical directional terms	3
2.1.2 Anatomical rotational terms	3

Introduction

This document contains my notes from Swansea University's graduate entry medicine course.

0.1 Organisation

The main body of the handbook is divided into chapters. Each chapter is stored as a separate file inside the `chapters` folder.

For example:

- `chapters/02-cardiovascular.tex`
- `chapters/03-respiratory.tex`
- `chapters/04-gastrointestinal.tex`

The chapters are imported in the required order from `main.tex`. Keeping one topic in each file makes the project easier to edit and reduces the chance of merge conflicts when several people contribute.

0.2 Suggested chapter structure

A clinical topic might be organised under the following headings:

1. definition and overview;
2. relevant anatomy and physiology;
3. aetiology and risk factors;
4. pathophysiology;
5. presentation;

6. investigations;
7. management;
8. complications;
9. prognosis and follow-up; and
10. key revision points.

Not every topic will require every heading.

0.3 Formatting conventions

Several reusable boxes are provided by `formatting.sty`.

Key point

Use this box for important principles or material that should be remembered.

Clinical note

Use this box for clinical context, practical interpretation, or links between scientific knowledge and clinical practice.

Red flag

Use this box for potentially serious presentations, urgent considerations, or common safety errors.

Exam tip

Use this box for assessment advice, useful contrasts, and easily confused concepts.

Example definition

Use this box when introducing a particularly important term or classification.

Important terms can also be written with `\term{important term}`, producing **important term**. Generic drug names can be written with `\drug{drug name}`, producing *drug name*.

0.4 Sources and maintenance

Where possible, notes should identify the source and date of clinical recommendations. Guidance, diagnostic criteria, drug information, and reference ranges can change over time.

Each chapter can end with a last-updated marker:

```
\lastupdated{2 September 2026}
```

Appropriate use

These notes must not contain patient-identifiable information. They are not a substitute for local protocols, current clinical guidelines, formal prescribing resources, or advice from an appropriately qualified professional.

CHAPTER 1

Administration

This chapter contains all of the initial administration processes I needed to do, and the order in which I did them.

1.1 Important links

Student feedback https://scs.swan.ac.uk/gem/feedback/students/feedback_index.php

Leave of absence request https://scs.swan.ac.uk/gem/isr/students/isr_loa.php

1.2 Registering with Learning@Wales

1. Go to learning.nhs.wales.
2. Select **Learning@Wales**.
3. Select the button right below the text that says

‘Please click the button below to register for an account if you are unable to self-register.’

1.3 Required training

Complete the following training modules:

- [Information Governance, Records Management & Cyber Security](#)
- [Fire Safety](#)
- [Infection Prevention & Control Level 1](#)

- Health & Safety
- Treat Me Fairly
- Safeguarding People
- Violence Against Women
- Violence and Aggression
- Manual Handling
- ANTT

1.4 Canvas training

- NAC - Anti-racism Course
- NAC - Enhance your EDI Skills

1.5 Resources

- Dissectible me, 5 minute anatomy
- Moore's clinically oriented anatomy
- Gray's anatomy for students
- 'kenhub'
- 'teaching anatomy'

CHAPTER 2

Anatomy

2.1 2026-09-03

2.1.1 Anatomical directional terms

- Superior
- Inferior
- Cephalic (towards the head)
- Caudal (towards the tail)¹
- Proximal (closer to the point of attachment)
- Distal (further from the point of attachment)
- Medial
- Lateral
- Posterior/dorsal
- Anterior/ventral

2.1.2 Anatomical rotational terms

- Supination: rotation of the forearm so that the palm faces anteriorly (or superiorly when the arm is outstretched)
- Pronation: rotation of the forearm so that the palm faces posteriorly (or inferiorly when the arm is outstretched)
- Inversion: rotation of the foot so that the sole faces medially
- Eversion: rotation of the foot so that the sole faces laterally

¹The terms **cephalic** and **caudal** are often used in embryology, where the head and tail of the embryo are more distinct than in adults.

- Flexion: movement that decreases the angle between two body parts²
- Extension: movement that increases the angle between two body parts²
- Abduction: movement away from the midline of the body
- Adduction: movement towards the midline of the body
- Circumduction: circular movement of a body part, combining flexion, extension, abduction, and adduction
- Rotation: movement around a longitudinal axis, either medially (internal rotation) or laterally (external rotation)
- Elevation: movement of a body part superiorly
- Depression: movement of a body part inferiorly
- Protraction: movement of a body part anteriorly
- Retraction: movement of a body part posteriorly
- Opposition: movement of the thumb to touch the tips of the other fingers
- Reposition: movement of the thumb back to its anatomical position

² the 'angle' being referred to here is the angle in the 'natural' movement (e.g. at the hip it is the raising of the leg that decreases the angle on the anterior side, which is the side that is usually considered when describing flexion and extension).